

Patient ID: NancyPeriod: June 22 to July 22, 2024Report Date: July 23, 2024Coverage: 417/738h (56%)

Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Clinical Alerting Thresholds

Very Low HR < 40 bpm

Very High HR > 110 bpm

Low HR < 45 bpm

Elevated Breathing > 24 brpm

Elevated HR > 95 bpm

High Breathing > 30 brpm

High HR > 100 bpm

Very High Breathing > 40 brpm

Last 24 Hours

Elevated Breathing 10 PM to 11 PM		Low HR 10 PM to 11 PM	
Vital Stat	Avg	Min	Max
Heart Rate	50 bpm	40 bpm	82 bpm
Breathing	17 brpm	9 brpm	27 brpm

Last 24 hours: four low heart rate episodes in the evening.

Episodic Events (last 24h)

Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
10 PM to 11 PM	4h	4	Elevated Breathing	17	9	27	Check SpO2 and lung sounds; assess for fluid overload or early infection
10 PM to 11 PM	4h	4	Low Heart Rate (brief)	44	40	49	Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic

Patient clinical status over 31 days from Jun 22 to Jul 22

■ HR ■ Breathing

Elevated Breathing Jun 23 to Jun 24 (2d)	Elevated Breathing Jul 04 to Jul 04 (1d)	Low HR Jul 21 to Jul 22 (2d)
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10 episodic events Detected, spanning 10h total. Conditions: Elevated Breathing, Low Heart Rate. Priority grade: Low

Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
10	10h	Low Heart Rate	<1	56%

Major Findings: Stable through mid period; end of monitoring Low Heart Rate clustering (Jul 21 to Jul 22, 1.0/day).

Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
Jul 21	3 AM to 4 AM	7h	7	Low Heart Rate	44	40	49	Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
Jun 23	3 PM to 4 PM	2h	2	Elevated Breathing	25	19	28	Check SpO2 and lung sounds; assess for fluid overload or early infection
Jul 04	10 PM to 11 PM	1h	1	Elevated Breathing	25	25	25	Check SpO2 and lung sounds; assess for fluid overload or early infection

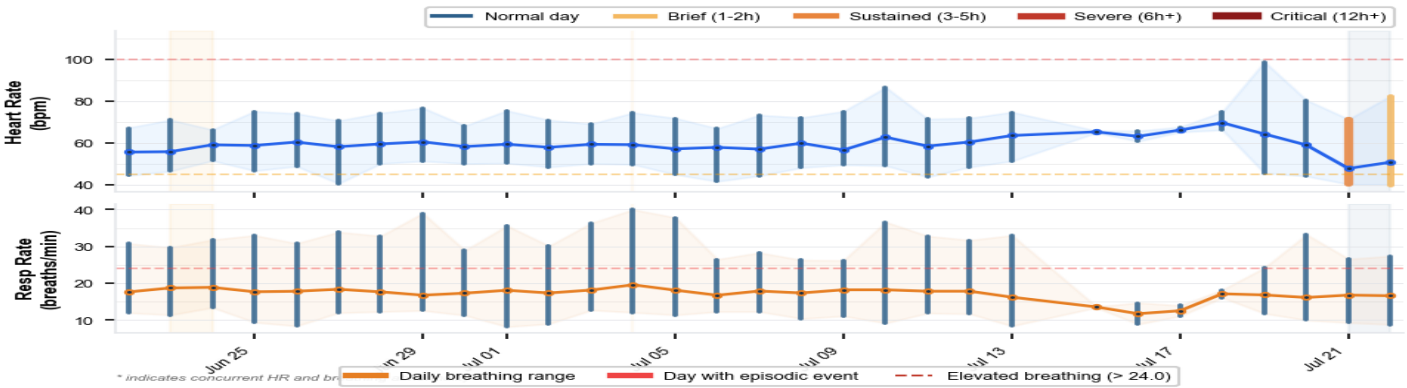
The P5 to P95 heart rate spread was 21 bpm (48 to 69), indicating significant cardiac variability.

Suggested Clinical Review Actions

- Low Heart Rate (avg 44 bpm, min 40 bpm): Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
- Elevated Breathing (avg 25 brpm, peak 28 brpm): Check SpO2 and lung sounds; assess for fluid overload or early infection
- Overall trajectory shows 3 unstable phases with 21 bpm variability. Consider increasing monitoring frequency and lowering escalation threshold.

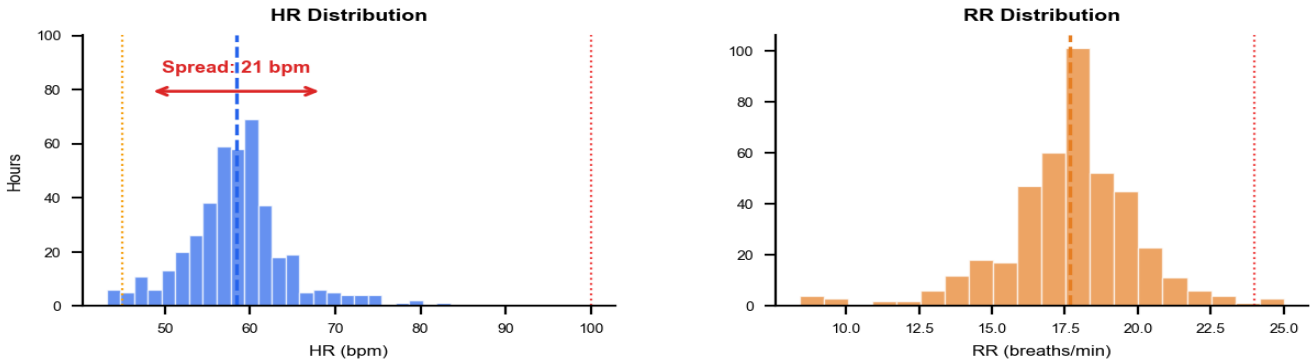
Clinical Pattern Observations:

- 1. **High variability:** HR spread 20 bpm (47 to 68).
- 2. **Clustered pattern:** Episodic events on 2 of 31 days (6%).
- 3. **Nocturnal pattern:** 4 of 6 eligible episodes during evening or night hours.

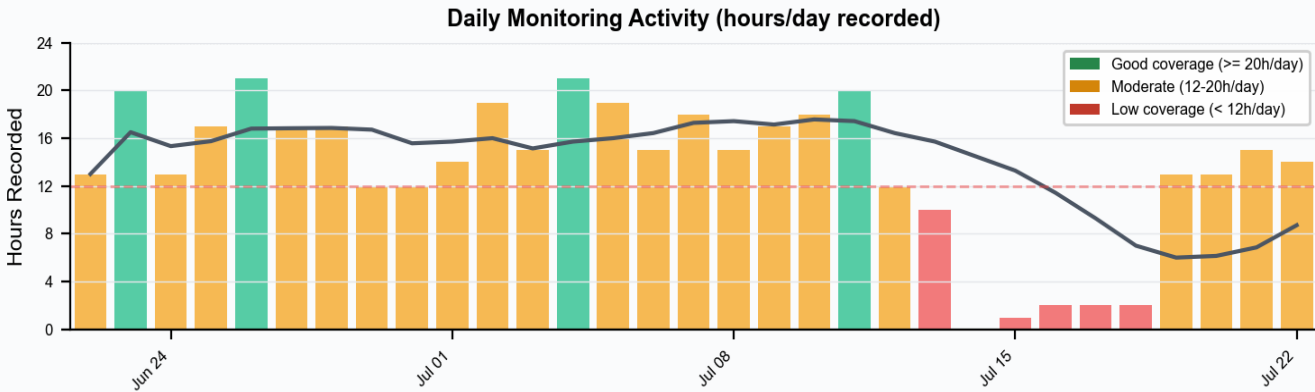


Red bars indicate days with detected episodic events.

Vital Sign Distribution



Monitoring Activity



Bars indicate monitoring hours/day. Red threshold line indicates clinical monitoring baseline.

■ HR episode ■ RR episode ■ Recorded, no episode ■ No data

Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.